

- For improvement of myotonic symptoms -

***Myonal*[®] 50mg**

TABLETS

<Eperisone hydrochloride preparation>

【COMPOSITION】

【PRODUCT DESCRIPTION】

Brand name	Dosage form and identification code	Appearance			Description
		Face	Reverse	Lateral	
MYONAL Tablets 50mg	Sugar –coated tablets				White
		Diameter (mm) 7.5	Weight (mg) 162	Thickness (mm) 4.2	

- Improvement of myotonic conditions caused by the following diseases:
Neck-shoulder-arm syndrome, scapulohumeral periarthritis and low back pain

- ## 【DOSAGE AND ADMINISTRATION】

【CONTRAINDICATIONS】

【PRECAUTIONS】

- (1) Patients with a history of drug hypersensitivity
- (2) Patients with hepatic function disorder
[MYONAL may aggravate hepatic function.]

Weakness, light-headedness, sleepiness or other symptoms may occur. In the event of such symptoms, the dosage should be reduced or treatment discontinued. Patients should be cautioned against engaging in potentially hazardous activities requiring alertness, such as operating machinery or driving a car.

Precautions for coadministration (MYONAL should be administered with care when coadministered with the following drugs.)

Drugs	Signs, Symptoms, and Treatment	Mechanism and Risk Factors
Methocarbamol	It has been reported that disturbance of visual accommodation occurred after the concomitant use of methocarbamol with tolperisone hydrochloride, an analogue compound.	Mechanism unknown

Since the elderly often have a physiological hypofunction, it is advisable to take measures, such as reduction in dosage under careful supervision.

1. Use During Pregnancy
MYONAL should only be used in pregnant women or women suspected of being pregnant, if the expected therapeutic benefits are evaluated to outweigh the possible risk of treatment.
[The safety of MYONAL in pregnant women has not been established.]

It is advisable to avoid the administration of MYONAL to nursing mothers. When MYONAL must be used, breast feeding should be discontinued during treatment.

[It has been reported that MYONAL is excreted in breast milk in an animal study (in rats).]

Safety in children has not been established (insufficient clinical experience).

Adverse reactions were reported in 416 of 12,315 patients (3.38%).
(At the end of the reexamination period)

(1) **Shock and anaphylactoid reactions**
Since shock and anaphylactoid reactions may occur, patients should be carefully observed. In the event of symptoms such as redness, itching, urticaria, edema of the face or other parts and dyspnea etc., treatment should be discontinued and appropriate measures should be taken.

Serious dermatopathy such as oculo-muco-cutaneous syndrome (Stevens-Johnson syndrome) or toxic epidermal necrolysis (Lyell syndrome) may occur. Patients should be carefully observed, treatment discontinued and appropriate measures taken, in the event of symptoms such as fever, erythema, blistering, itching, ocular congestion or stomatitis, etc.

	5%> ≥ 0.1%	<0.1%	Incidence unknown
Hepatic ^{note1)}		Elevation of AST (GOT), ALT(GPT) and Al-P, etc.	
Renal ^{note1)}		Proteinuria and Elevation of BUN, etc.	
Hematologic ^{note1)}		Anemia	
Hypersensitivity ^{note2)}	Rash	Pruritus	Erythema exudativum multiforme
Psychoneurologic	Sleepiness, insomnia, headache and numbness in the extremities	Stiffness and tremor in the extremities	
Gastrointestinal	Nausea/vomiting, anorexia, stomach discomfort, abdominal pain, diarrhea, constipation and thirst	Stomatitis and feeling of enlarged abdomen	
Urinary		Urinary retention, urinary incontinence and feeling of residual urine	
General	Weakness, light-headedness and generalized fatigue	Muscle hypotonia and dizziness	
Others	Hot flushes	Diaphoresis, edema and palpitations	Hiccup

Note 2) In the event of such symptoms, treatment should be discontinued.

Blood concentration

Figure 1 consists of three line graphs showing plasma concentration (ng/mL) versus time (hr) for Day 1, Day 8, and Day 14. Each graph shows a peak concentration around 1-2 hours, with error bars indicating variability. The concentration decreases over time, reaching near zero by 6 hours.

Day	Time (hr)	Plasma Concentration (ng/mL)
Day 1	0	0
	1	2.0
	2	6.5
	3	4.5
	4	2.5
	5	1.5
	6	0.5
Day 8	0	0
	1	2.5
	2	5.5
	3	4.5
	4	3.0
	5	1.5
	6	0.5
Day 14	0	0
	1	2.0
	2	6.5
	3	4.5
	4	2.5
	5	1.5
	6	0.5

Note) A single dose of 150 mg is unapproved.

1. Skeletal muscle relaxation

- ## 2. Vasodilatation and Augmentation of blood flow

- ### 3. Analgesic action and inhibition of the pain reflex in the spinal cord

4. Facilitation of voluntary movement

【CLINICAL STUDIES】

In open labeled clinical trials and a double blind controlled clinical trial undertaken to determine the effects of MYONAL on myotonic symptoms associated with these diseases, an efficacy rate of 52.1% (234/449) was achieved. (When fairly effective responses are included, the efficacy rate was as high as 80.4 %.)

In open labeled clinical trials and a double blind clinical trial, the usefulness of MYONAL has been established for spastic paralysis associated with diseases such as cerebrovascular disturbances, spastic spinal paralysis or cervical spondylosis. Improvement rates for rigidity and stiffness in patients with spastic paralysis were 42.3% (197/466) and 45.1% (174/386), respectively.

It contains carnauba wax, carmellose, microcrystalline cellulose, titanium oxide, stearic acid, calcium stearate, sucrose, talc, precipitated calcium carbonate, corn starch, white shellac, hydroxypropylcellulose, pullulan, povidone K30, macrogol 6000 and hydrated silicon dioxide as inactive ingredients.

MYONAL should be used before the expiration date indicated on the package or label.

MYONAL should be stored below 30°C.
MYONAL should be protected from moisture after opening package.

Boxes of 100 in press-through packages

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